



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض رابعياً : ساميه سعدالدين عثمان -

المحافظة : القاهرة الحي / المدينة : مدينه نصر الاول العنوان : ----

رقم البطاقة : 25412080104627 رقم التليفون : 01006790844 تاريخ تسجيل الإستمارة : 03-10-2026

العمر (تاريخ الميلاد) : 08-12-1954 المهنة : بدون عمل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... /د. /..... /د. /..... /د. /..... /د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: ساميه سعدالدين عثمان - National ID: 25412080104627	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

سامية
سعد الدين عثمان

فيلا ١٩٠ منطقة ٢ البلدي
التجمع الخامس - القاهرة

١٩٥٤/١١  ٢٥٤ ١٢ ٠٨ ٠١ ٠٤٦ ٢٧

LP0719443

٢٠٢٦/٠١ ٢٥٤١٢٠٨٠١٠٤٦٢٧ 

بدون عمل

أنثى متعلمة متزوجة

محمد احمد نور الدين عبدالغفار

البطاقة سارية حتى ٢٠٢٢/٠١/٢٥

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Patient Name : الأستاذة / سامية سعد الدين عثمان

Patient No. : 25412080104627

Birth No. : 4012/23

Sex : Female

Age : 68 Y

Referring Doc. :

Request No. : 13002845

Request Date : 13/11/2023

Verification Date : 22/11/2023

Financial Category : اورام دار السلام - نفقة الدولة

Department : Out Patient

IHC No. :

Registered by : Ali mostafa

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast lesion.

Nature of Specimen: Tru-cut biopsy, collected at 13/11/2023, 9:45 am.

Technique:

Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:

ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

PT (Positive control showed appropriate reactivity)

ER: Strong nuclear reaction in 90% tumor cells (Allred score 8/8).

PR: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8).

Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).

Ki67: 35% nuclear reaction of tumor cells.

Conclusion:

Right breast lesion, Tru-cut biopsy:

Invasive duct carcinoma, Grade II with micro papillary component.

T:

ER: Positive (score 8/8).

PR: Positive (score 8/8).

Her2: Positive (score 3+).

Ki67: Positive in 35% of tumor cells.

Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and

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DR.Menna Nabil

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Patient Name	الأسنان / سامية سعد الدين عثمان	Request No.	13002845
Patient No.	25412080104627	Request Date	13/11/2023
Path No.	4012/23	Verification Date	22/11/2023
Sex	Female	Financial Category	اورام دار السلام - نفقة الدولة
Age	68 Y	Department	Out Patient
Referring Doc.		Registered by	Ali mostafa

IMMUNOHISTOCHEMISTRY REPORT

, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

Scoring system: "Her2".

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Prof. Dr. Amal Hareedy.

Dr. Heba Hassan.

Dr. Sagda Elkady.

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DR.Menna Nabil

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



بجينة جديدة
للفحولة

Patient Name : الأستاذ / ساميه سعد الدين عثمان	Request No. : 13008436
Patient No. : 25412080104627	Request Date : 15/05/2024
Path No. : 1586/24	Verification Date : 23/05/2024
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 69 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Right breast cancer post NACT .
Nature of the specimen: Right breast & Right axilla (MRM)

Gross Examination:

- Procedure: Right MRM
- measured 23x14x4 cm with covering skin ellipse measured 17x12cm .
- Localization: UOQ
- Tumor extent : A greyish white tumour bed site
- Size: 1.5 x1.3 x1.3 cm .
- Skin involvement: Nipple & areola were unremarkable grossly .
- Distance from margins: 1.5 cm away from deep margin
- Microscopic examination:
- Histological type: Invasive duct carcinoma (Actual tumour size 1.5x1.3 cm , Cellularity 30%)
- Grade (differentiation+ nuclear pleomorphism + mitosis): G II(3+2+1)
- Ductal carcinoma in situ: Focal (2 %)
- Skin involvement: Free nipple and areola
- Lymph node involvement: Positive L.Ns (1/21) with extracapsular extension
- Resection Margins: Free margins
- Lymphovascular emboli: Positive
- Perineural invasion: Positive
- Histologic response to therapy: Pathologic partial response

Diagnosis:

- Right MRM (post NACT) :
Invasive duct carcinoma grade II
- Ductal carcinoma insitu
 - Free margins

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Patient Name : الأستاذة / سامية سعد الدين عثمان	Request No. : 13008436
Patient No. : 25412080104627	Request Date : 15/05/2024
Path No. : 1586/24	Verification Date : 23/05/2024
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 69 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

- Free nipple and areola
- Positive L.Ns (1/21)
- Pathologic partial response
- Stage : ypT1, ypN1
- RCB : 2.867 , RCB class II

Dr. Rana Fathy

Signature
Dr. Walaa Ghoneim

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Patient Name : الأستاذ / سامية سعد الدين عثمان	Request No. : 13002845
Patient No. : 25412080104627	Request Date : 13/11/2023
Path No. : 4012/23	Verification Date : 20/11/2023
Sex : Female	Financial Category : نفقة الدولة - أورام دار السلام
Age : 68 Y	Department : Out Patient
Referring Doc :	Registered by : Ali mostafa

Pathology Report

Clinical Data: Right breast lesion.

Nature of Specimen: Tru-cut biopsy, collected at 13/11/2023, 9:45 am.

Gross Examination:

- Multiple grayish white tissue cores up to 2 cm, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: No.
- * Perineural invasion: No.
- * Angioinvasion: No.
- * Additional pathological finding: Micro papillary component.

Diagnosis:

Right breast lesion, Tru-cut biopsy:

Invasive duct carcinoma, Grade II with micro papillary component.

For further IHC (ER, PR, Her2, Ki67).

Signature

Prof. Dr. Amal Hareedy.

Dr. Heba Hassan.

Dr. Sagda Elkady.

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / سامية سعد الدين عثمان	Request No. : 13002845
Patient No. : 25412080104627	Request Date : 13/11/2023
Path No. : 4012/23	Verification Date : 20/11/2023
Sex : Female	Financial Category : أورام دار السلام - نفقة الدولة
Age : 68 Y	Department : Out Patient
Referring Doc :	Registered by : Ali mostafa

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- * Angioinvasion: No.
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Diagnosis:

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For further IHC (ER, PR, Her2, Ki67).

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NAME: SAMIA SAAD ALDIN OSMAN	PATIENT ID: 719720
DEAR SIR	STUDY DATE: 23/04/2024
Acc No: 1596982	

¹⁸F-FDG PET/CT EXAMINATION

History: -

68-year-old female patient, presented with history of right breast cancer, received chemotherapy and targeted therapy, referred for follow-up PET/CT examination.

Procedure: -

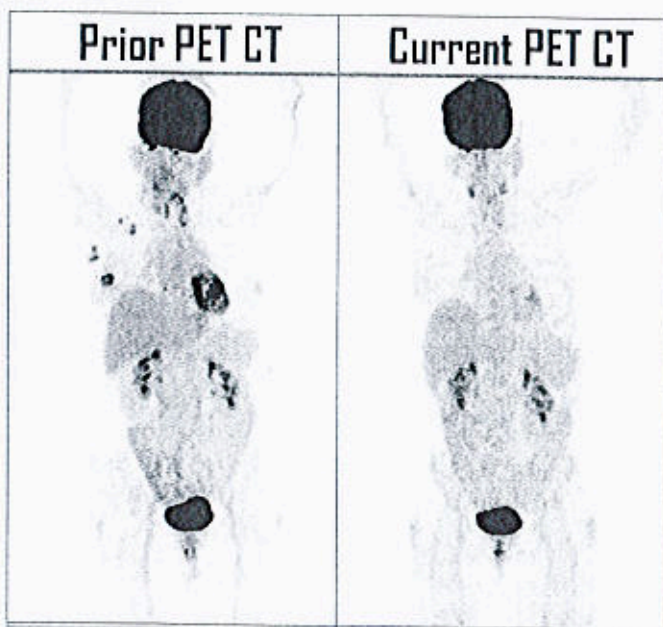
- 6.8 mCi ¹⁸F-FDG was administrated to the patient and imaged after appropriate time.
- The blood glucose level was 82 mg/dl.
- A low dose non-contrast CT for attenuation correction and anatomic localization followed by PET images from the skull vault to the mid-thigh were obtained.
- A diagnostic 128 MDCT post contrast examination was taken after I.V. non-ionic contrast administration of the same regions. Images of CT and corresponding functional PET images are taken in axial, coronal and sagittal planes.

PET/CT FINDINGS: -

Compared to the previous PET/CT study dated 21/11/2023, the current examination revealed:

A) Head and Neck: -

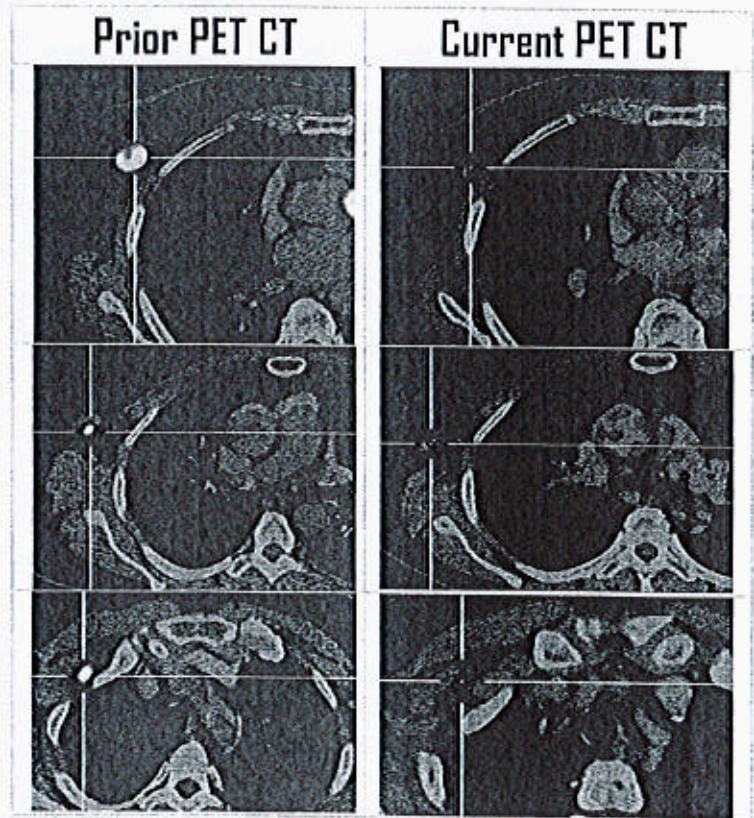
- The neck is devoid of any hypermetabolic lymph nodes.
- The visualized portions of the brain exhibited normal FDG biodistribution.
- Physiological FDG uptake is seen in the oropharynx, salivary glands and larynx.
- The thyroid gland is still showing mild diffuse increased FDG uptake, currently achieving up to 4.46 SUVmax (compared to 5.9 SUV max previously), with left lobe hypodense nodule is seen, measuring 10 mm.



NAME: SAMIA SAAD ALDIN OSMAN	PATIENT ID: 719720
DEAR SIR	STUDY DATE: 23/04/2024
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B) Chest: -

- **The right breast shows remarkable metabolic morphological regression** of the previously noted UOQ hypermetabolic heterogeneously enhancing soft tissue mass lesion, currently measuring +/- 15x8 mm and achieving up to 2.3 SUVmax (compared to 24x15 mm & 10.15 SUVmax previously). **Complete metabolic resolution** of the associated tiny perilesional satellites, measuring up to 6 mm.
- **Complete metabolic resolution** of the previously detected multiple hypermetabolic right axillary and sub pectoral lymph nodes are seen, currently measuring up to 12x7mm at axillary group (compared to 20x12 mm previously).
- **Complete metabolic resolution** of another right internal mammary mildly active lymph node.
- No newly developed hypermetabolic lymphadenopathy seen.
- Still no left breast hypermetabolic lesion seen.
- No hypermetabolic pulmonary nodules detected.
- Newly developed right middle lobe inactive consolidative zone; *mostly inflammatory*.
- No pleural effusion seen.
- Normal tracer biodistribution uptake is noted in the myocardium.



NAME: SAMIA SAAD ALDIN OSMAN	PATIENT ID: 719720
DEAR SIR	STUDY DATE: 23/04/2024
Acc No: 1596982	

C] Abdomen & Pelvis: -

- Physiological uptake in liver, adrenal glands, spleen and pancreas
- There is normal distribution of the radiotracer within the gastrointestinal and genitourinary systems without focal areas of abnormal metabolism.
- Uterine posterior wall sub serous inactive calcified fibroid is still noted.
- Multiple small noncomplicated colonic diverticula are seen.
- Physiological excretion of FDG in the kidneys.
- No ascites or hypermetabolic lymphadenopathy noted.

D] Musculoskeletal: -

- No metabolically active osseous lesions could be detected along the surveyed skeleton.
- Spondylodegenerative changes are seen affecting multiple levels of spine.

CONCLUSION: -

Follow-up PET/CT study revealed:

- Remarkable metabolic and morphological regression of the right breast UOQ primary malignant neoplasia.
- Complete metabolic resolution of the associated hypermetabolic right axillary, sub pectoral and right internal mammary nodal deposits.
- No newly developed metabolically active lesion detected.

MUCH OBLIGED

SHAIMAA SHAWKY, MD

A.PROF.MOSTAFA ABDELKAWY